Chamundi Childrens Hospital Laboratory

JLB Road, Mysuru 570005

DEPARTMENT OF LABORATORY MEDICINE · PAEDIATRICS

Patient Name	**Kiran Gowda Shivakumar**	Accession No.	**CHM/2021/03137**
Hospital No	**CHM/384051**	Sample Collected	**18/10/2021 9:40 AM**
Age / Sex	**9 Y / M**	Report Released	**18/10/2021 5:20 PM**
Referred By	**Dr. Ramesh Shetty, MBBS, MD (Paediatrics)**	Specimen	**Whole blood (EDTA), Serum**

COMPLETE BLOOD COUNT (CBC)

INVESTIGATION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Haemoglobin	10.8 L	g/dL	11.5 - 15.5
RBC Count	5.42 H	mil/uL	4.0 - 5.2
PCV	34.6 L	%	35 - 45
MCV	63.8 L	fL	77 - 95
MCH	19.9 L	pg	24 - 30
MCHC	31.2 L	g/dL	31.5 - 34.5
RDW-CV	14.2 H	%	11.6 - 14.0
Total WBC Count	7400	/uL	4500 - 13500
Neutrophils	48	%	35 - 65
Lymphocytes	42	%	30 - 55
Monocytes	6	%	2 - 10
Eosinophils	3	%	1 - 6
Basophils	1	%	0 - 2
Platelet Count	318000	/uL	150000 - 450000

HAEMOGLOBIN STUDIES

INVESTIGATION	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
HbA2 (HPLC)	5.2 H	%	2.2 - 3.5
HbF (HPLC)	1.4	%	< 2.0
Ferritin	58	ng/mL	7 - 140

HbA2 - beta thalassaemia trait

HbA2 fraction	Interpretation
2.2 - 3.5 %	Normal
3.6 - 4.0 %	Borderline, repeat with iron studies
Above 4.0 %	Consistent with beta thalassaemia trait

Interpretation

Microcytosis out of proportion to the haemoglobin, with normal iron stores and a raised HbA2. This is beta thalassaemia trait; iron therapy is not indicated and should be stopped.

Tested by: S. Pawar (B.Sc. MLT)

Results released electronically. Verified before release.

S Rao

Dr. Shakuntala Rao

MD, DNB (Pathology)

Regn. No. KMC 2003/06/1926